

Our Ref: KS /TS/ [MED-ID]

[DATE]

Clinic Letter : [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

Totton, Southampton

Hampshire

[POSTCODE]

Division of Unscheduled Care

Care Group - Medicine & Elderly Care Gastroenterology

CF90, F Level, Centre Block,, Mailpoint

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]Ext

Dear [PROVIDER-NAME] [NAME] , [ADDRESS]. [POSTCODE] DOB: [DATE] , Hospital Number: [MED-ID] ,

[NHS-NO]

Diagnoses:

Ulcerative colitis

Sub-total colectomy with ileoanal pouch

Previous defunctioning ileostomy with re-anastomosis early 2008

Short bowel syndrome

Bilateral DVT's on long-term warfarin

Partial hypoadrenalism

Epilepsy

Current Medications: Warfarin, Loperamide 13 tablets a day, Dihydrocodeine two tablets four times a day, 1 litre of St Mark's solution per day, Metronidazole 400 milligrams once daily (pouchitis prophylaxis), Omeprazole 20 milligrams once daily, Alendronic acid once weekly, Citalopram, Calcichew D3.

I saw MISS [NAME] in the Nutrition Clinic with my nutrition support colleagues today. She is [NAME] herself. Her weight has gone up and she now weighs 108.3 kg. She is having her bowels open about 3-4 times a day, which is normal for her. She does, however, get recurrent pouchitis and although we have put her on Ciprofloxacin prophylactically, she has recently had recurrence of pouchitis and now has to go back onto Metronidazole. She is now on Metronidazole 400 milligrams once daily as a prophylaxis. She is eating well and drinks her 1 litre of St Mark's solution everyday. In addition to this she has four cups of coffee or tea and about a pint of soda and lime during the night when she gets very thirsty. She is passing good amounts of urine and her recent urinary sodium was 49. The rest of her blood tests are reasonable, although her haemoglobin is 109 with a transferrin saturation of 9%, which is low for her and was previously.

I have advised her to try and reduce the amount of soda and lime she drinks during the night and this may paradoxically help with her dry mouth. I would be grateful if you could prescribe iron sulphate tablets to take over the next three or four months. She is going to have blood tests before she comes back next in four months' time and at that point we will need to organise for her to have a DEXA scan. We will also organise for her to be reviewed by [NAME] our dietitian.

With many thanks

Yours sincerely

[PROVIDER-NAME]

Specialist Registrar in Gastroenterology